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Herpes zóster

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Actividad de formación continua

Esta actividad de formación médica continua revisa la presentación clínica, el diagnóstico, el tratamiento, la prevención y las complicaciones del **herpes zóster** , una reactivación del virus varicela- **zóster** latente que puede causar dolor agudo intenso, neuralgia postherpética, enfermedad ocular, complicaciones neurológicas e infección diseminada. La actividad aborda una carencia en la práctica clínica en cuanto al reconocimiento oportuno de las lesiones dermatomal características, las pruebas diagnósticas adecuadas cuando las presentaciones son atípicas, la terapia antiviral temprana, el tratamiento del dolor, la derivación a oftalmología y el asesoramiento sobre vacunación. Los participantes **revisarán** las características clínicas que distinguen **el herpes zóster** de afecciones dermatológicas, orales, neurológicas y oculares que lo imitan. La actividad también examina la dosificación antiviral basada en la evidencia, las estrategias para la neuralgia postherpética, la evaluación del riesgo en adultos mayores y pacientes inmunocomprometidos, y la prevención mediante la vacunación recombinante **contra el zóster** . Se espera que los participantes mejoren la precisión diagnóstica, seleccionen la terapia adecuada, reduzcan las complicaciones prevenibles y coordinen la atención centrada en el paciente en todo el equipo interdisciplinario.

Objetivos:

- Identificar las características clínicas que favorecen el diagnóstico precoz del **herpes zóster** en pacientes con dolor dermatomal, lesiones vesiculares o presentaciones atípicas.
- Diferenciar **el herpes zóster** de la infección por el virus **del herpes** simple, la dermatitis de contacto, el impétigo, las picaduras de insectos, los trastornos ampollosos orales y otras afecciones que pueden presentar síntomas similares.
- Evaluar los factores de riesgo de neuralgia postherpética, complicaciones oculares, afectación neurológica, enfermedad diseminada y dolor intenso.
- Coordinar la atención entre médicos de atención primaria, farmacéuticos, enfermeros, especialistas en dolor y oftalmólogos para pacientes con presentaciones de **herpes zóster** de alto riesgo .

Acceda a preguntas de opción múltiple gratuitas sobre este tema.

Introducción

El virus varicela- **zóster** (VVZ), también conocido como herpesvirus humano 3, es el agente causal de la varicela y el **herpes zóster** . El VVZ es un herpesvirus humano neurotrópico **perteneciente** a la subfamilia Alphaherpesvirinae. [1] **El herpes zóster** es una enfermedad viral causada por la reactivación del VVZ, que permanece latente en los ganglios sensitivos de los nervios craneales o en los ganglios de la raíz dorsal tras una infección previa por varicela. La latencia

del VVZ está controlada principalmente por la inmunidad celular, y se cree que la reactivación se debe a la pérdida de esta vigilancia inmunitaria. [2] [3] [4]

La incidencia del **herpes zóster** está estrechamente relacionada con el estado inmunitario. Las personas con un sistema inmunitario fuerte rara vez desarrollan herpes zóster. Incluso después de que **el herpes zóster** remita, muchos pacientes siguen experimentando dolor moderado a intenso, conocido como neuralgia postherpética.

Etiología

Tras su reactivación, el virus se replica en los cuerpos celulares neuronales, y los viriones se liberan de las células y son transportados a través del nervio hasta la zona de la piel inervada por ese ganglio. En la piel, el virus provoca inflamación local y ampollas. El dolor causado por el **herpes zóster** se debe a la inflamación de los nervios afectados, asociada al virus. [5] [6] [7]

Los factores desencadenantes del **herpes zóster** incluyen:

- Estrés emocional
- Medicamentos inmunosupresores
- Enfermedad aguda o crónica
- Reexposición al virus varicela **zóster**
- Presencia de una neoplasia maligna

Epidemiología

La incidencia del **herpes zóster** oscila entre 1,2 y 3,4 casos por cada 1000 personas al año en individuos jóvenes y sanos, mientras que en pacientes mayores de 65 años la incidencia es de 3,9 a 11,8 casos por cada 1000 personas al año. **El herpes zóster** no presenta variación estacional. Las recurrencias son más frecuentes en personas inmunodeprimidas.

Fisiopatología

Las lesiones cutáneas del **herpes zóster** inducen la proliferación de linfocitos T específicos del virus varicela- **zóster , mientras que la producción de interferón α conduce a la resolución del herpes zóster** . En pacientes inmunocompetentes, los anticuerpos específicos (IgG, IgM e IgA) se producen más rápidamente y alcanzan títulos más altos durante la reactivación (**herpes zóster**) que durante la infección primaria, lo que da lugar a una inmunidad celular mejorada y duradera contra el virus varicela- **zóster** .

The cutaneous presentation is centripetal and follows a dermatome. In most cases, the lumbar and cervical roots are involved, whereas motor involvement is rare. The infection is contagious to individuals with no prior immunity to the varicella-**zoster** virus; however, transmission rates are low. The virus can be transmitted either through direct skin contact or by inhaling infected droplets. Furthermore, concurrent herpesvirus infections can also occur. **Herpes** simplex virus, cytomegalovirus, Epstein-Barr virus, and human herpesviruses have all been identified in patients with shingles.

History and Physical

Zoster characteristically presents with a prodrome of fever, malaise, and excruciating burning pain, followed by the outbreak of vesicles that appear in 1 to 3 crops over 3 to 5 days. Lesions are distributed unilaterally within a single dermatome (see **Image**. **Herpes Zoster**). Clinically, lesions start as closely grouped erythematous papules that rapidly become vesicles on an erythematous and edematous base and may occur in continuous or interrupted bands in 1, 2, or

more contiguous dermatomes unilaterally (see **Image**. **Herpes Zoster** Rash). Commonly involved dermatomes include the thoracic (53%), cervical (20%), trigeminal (15%), and lumbosacral (11%). **Herpes zoster** can also present with ophthalmic involvement.

The 3 phases of the infection include:

- **Preeruptive stage**: The preeruptive stage presents with abnormal skin sensations or pain within the affected dermatome. The preeruptive stage occurs at least 48 hours before any obvious lesions appear. At the same time, the individual may experience headaches, general malaise, and photophobia.
- **Acute eruptive phase**: The acute eruptive phase is marked by the vesicles and the symptoms seen in the preeruptive phase (see **Image**. **Herpes Zoster** Vesicles). The lesions initially appear as macules and quickly progress to painful vesicles. The vesicles often rupture, ulcerate, and eventually crust over. Patients are most infectious in this stage until the lesions dry out. Pain is severe during this phase and often unresponsive to traditional pain medications. The phase may last 2 to 4 weeks, but the pain may continue.
- **Chronic infection**: Chronic infection is characterized by recurrent pain lasting more than 4 weeks. In addition to pain, patients experience paresthesias, shock-like sensations, and dysesthesias. The pain is disabling and may last 12 months or longer.

Herpes zoster has specific presentations when the ear and mouth are involved. For example, shingles oticus is also known as Ramsay Hunt syndrome type 2. This condition results from the virus spreading from the facial nerve to the vestibulocochlear nerve, which innervates the inner ear, causing hearing loss and vertigo (rotational dizziness). Moreover, **zoster** may occur in the mouth if the maxillary or mandibular division of the trigeminal nerve is affected. Clinically, oral **zoster** presents with vesicles or erosions occurring over the mucous membrane of the upper jaw (palate and gums of the upper teeth) or the lower jaw (tongue or gums of the lower teeth). Oral involvement may occur alone or in combination with the lesions on the skin over the cutaneous distribution of the same trigeminal branch. Because of the close relationship of blood vessels to nerves, the virus can spread to involve the blood vessels, compromise the blood supply, and cause ischemic necrosis. Complications such as osteonecrosis, tooth loss, periodontitis, pulp calcification, pulp necrosis, periapical lesions, and tooth developmental anomalies can result from oral **zoster**.

Herpes zoster can also involve the eye and central nervous system. The ophthalmic division of the trigeminal nerve is the most commonly involved branch, causing ophthalmic **zoster**. The skin of the forehead, upper eyelid, and orbital region may be involved (see **Image**. Causes of Epiphora). Ophthalmic **zoster** occurs in approximately 10% to 25% of cases and presents with features of keratitis, uveitis, and optic nerve palsies (see **Image**. Follicular Conjunctivitis). The Hutchinson eye sign is the presence of vesicles on the tip of the nose or nasal mucosa on the ipsilateral side of **herpes zoster** ophthalmicus infection, indicating involvement of the nasociliary branch of the ophthalmic division of the trigeminal nerve (see **Image**. **Herpes Zoster** Ophthalmicus With Hutchinson Sign). Notably, central nervous system involvement is not uncommon. Because the virus resides in the sensory root ganglia, VZV can affect any part of the brain, causing cranial nerve palsies, muscular weakness, diaphragmatic paralysis, neurogenic bladder, Guillain-Barré syndrome, and myelitis. In severe cases, patients may develop encephalitis.

Disseminated **zoster** is defined as more than 20 skin lesions developing outside the primarily affected area or dermatomes directly adjacent to it. In addition to the skin, other organs may also be affected, causing hepatitis or encephalitis, making this condition potentially lethal.[8] **Zoster** sine herpete is a condition characterized by radicular pain along a dermatomal distribution without the presence of an erythematous or vesicular rash.[9] Postherpetic neuralgia is the persistence of pain for 3 months after the onset of **herpes zoster**. Postherpetic neuralgia is the most common complication seen in older adults with involvement of the ophthalmic division of the trigeminal nerve. Risk factors for postherpetic neuralgia include advanced age, severe prodromal pain, severe pain during an attack, severe

rash, and immunosuppression. Finally, varicella may lead to infection in the fetus during pregnancy and complications in the newborn, but chronic infection or reactivation, known as **herpes zoster**, is not associated with fetal infection.

Evaluation

Herpes zoster is clinically diagnosed based on burning pain, characteristic morphology, and typical distribution. **Herpes** simplex virus can occasionally produce a rash in a pattern called zosteriform **herpes** simplex.[10][11][12]

Dermatoscopy of **herpes zoster** consistently shows the following:

- An erythematous background, indicating increased vascularity, is the most common finding and is seen in 100% of cases.
- Round, cloudy-white polyglobular structures representing grouped vesicles are observed in 53% to 80% of patients.
- Central brown dots are present in 50% to 80% of cases and likely correlate with the crusted centers of vesicular lesions (see **Image**. Dermoscopic Vesicles in **Herpes Zoster**).
- The distinctive halo sign appears due to the contrast between the yellowish or brownish crusted centers of vesicles and the surrounding erythematous background, creating an annular pattern around each element of the rash (see **Image**. Dermoscopic Halo Sign in **Herpes Zoster**).

Dermatoscopy plays a significant role in the early diagnosis of **herpes zoster**, enabling diagnosis approximately 1.62 days earlier than clinical evaluation alone. These dermoscopic features assist clinicians in differentiating **herpes zoster** from other similar dermatologic conditions and enable earlier initiation of treatment.[13]

Tests for varicella-**zoster** virus include the following:

- The Tzanck smear of vesicular fluid shows multinucleated giant cells. The Tzanck smear has lower sensitivity and specificity than direct fluorescent antibody testing or polymerase chain reaction (PCR).
- Varicella-**zoster** virus–specific IgM antibody in blood is detected during active infection with chickenpox or shingles but not when the virus is dormant.
- Direct fluorescent antibody testing of vesicular fluid or corneal fluid can be done when eye involvement is present.
- PCR testing can be performed on vesicular fluid, corneal lesions, or blood in patients with ocular involvement or disseminated infection.

Molecular biology tests based on in vitro nucleic acid amplification (PCR tests) are currently considered the most reliable. The nested PCR test has high sensitivity but is susceptible to contamination, which can lead to false-positive results. The latest real-time PCR tests are rapid, easy to perform, associated with a lower risk of contamination, and more sensitive than viral cultures.

Treatment / Management

Antiviral therapy hastens lesion resolution, reduces acute pain, and helps prevent postherpetic neuralgia, especially in older adults. The standard therapy for **herpes zoster** is acyclovir and its prodrug valacyclovir. Acyclovir and valacyclovir are processed to nucleoside analogues that specifically block viral DNA replication in affected cells. Oral bioavailability of acyclovir is 10% to 30%. Approximately 15% is metabolized in the liver, whereas 85% of acyclovir is excreted unchanged by the kidney through glomerular filtration and tubular secretion. Acute crystalline nephropathy is a reversible form of acute kidney injury caused by intratubular precipitation of acyclovir crystals and

obstruction; histology may show crystals in the distal tubules with patchy interstitial infiltration, without tubular necrosis.[14]

Mutations in the viral thymidine kinase or DNA polymerase confer acyclovir resistance. For patients with acyclovir-resistant **herpes zoster**, famciclovir is an alternative. Valganciclovir is a valine ester prodrug of ganciclovir that can be administered orally, and recent findings showed activity against varicella-**zoster** virus.

Doses:

- **Acyclovir**: 800 mg, 5 times daily for 5 days
- **Valacyclovir**: 1 g 3 times daily for 5 days
- **Famciclovir**: 500 mg 3 times daily for 7 days

In immunocompromised patients, intravenous acyclovir is administered at 10 mg/kg every 8 hours for 7 days. Ideal body weight should be used for patients with obesity. Intravenous infusion is preferred over 1 hour, using a diluted 5% dextrose in water solution or 0.9% sodium chloride, to a final concentration of 7 mg/mL or less at a constant rate to prevent renal damage. Please see StatPearls' companion reference, "Acyclovir," for further information.

Topical antibiotic creams such as mupirocin or framycetin help prevent secondary bacterial infection, and analgesics help relieve pain. Occasionally, severe pain may require an opioid medication. Topical lidocaine and nerve blocks may also reduce pain.[10][15][16]

Pharmacologically, first-line systemic treatments consist of anticonvulsants such as gabapentin and pregabalin. Pregabalin, in particular, may provide faster pain relief than gabapentin, but both require careful dose titration and monitoring for adverse effects such as sedation and cognitive impairment. Tricyclic antidepressants such as amitriptyline and nortriptyline are also commonly used because of their analgesic properties; nortriptyline tends to be better tolerated. Opioids such as tramadol are considered third-line because of concerns about adverse effects and dependency. These systemic agents often require combination therapy to optimize pain control while minimizing adverse events.[17][18]

Topical therapies include 5% lidocaine and 8% capsaicin patches. Lidocaine patches provide localized pain relief with minimal systemic effects, making them suitable for older adults and patients with comorbidities. Capsaicin patches, derived from chili peppers, desensitize peripheral nociceptors, with higher-concentration patches offering prolonged analgesia after a 30- to 90-min application under medical supervision.

Interventional pain treatment techniques such as the modified Jaipur block (local anesthetics combined with corticosteroids), spinal cord stimulation, and dorsal root entry zone lesioning have gained attention. These techniques are particularly helpful in refractory cases or for patients with severe neuropathic pain. The modified Jaipur block is a local subcutaneous infiltration of 2% lidocaine, 0.5% bupivacaine, and methylprednisolone. The involved dermatomes are identified, scarred areas are avoided, and areas of maximum pain are identified. At these sites, 2 mL is injected subcutaneously, with a maximum of 8-10 injections per sitting. In the original Jaipur block, the corticosteroid traditionally used was dexamethasone.[17] Additionally, nonpharmacological approaches are emerging adjunctive therapies, including frequency-specific microcurrent therapy that reduces burning and stabbing pain in patients with postherpetic neuralgia unresponsive to medications. Transcutaneous electrical nerve stimulation combined with other agents such as methylcobalamin has also demonstrated beneficial effects.

Prevention remains essential, with **herpes zoster** vaccination significantly lowering the incidence and severity of both **herpes zoster** and postherpetic neuralgia in immunocompetent older adults. The recombinant **zoster** vaccine (Shingrix; GlaxoSmithKline) is a subunit vaccine containing an adjuvant that the Advisory Committee on Immunization Practices recommended for use in adults 50 years or older in 2017. In October 2021, the Advisory Committee on Immunization Practices approved the recombinant **zoster** vaccine for preventing **herpes zoster** in

adults 19 years or older who are or will be at an increased risk of **herpes zoster** due to immunodeficiency or immunosuppression caused by known diseases or therapy.[19]

Differential Diagnosis

Cutaneous lesions of **herpes zoster** should be differentiated from **herpes** simplex, dermatitis herpetiformis, impetigo, contact dermatitis, candidiasis, drug reactions, and insect bites. Preceding pain without the development of skin lesions in **herpes zoster** should be differentiated from cholecystitis and biliary colic, renal colic, trigeminal neuralgia, and dental infection. **Herpes zoster** typically involves only 1 side of the oral cavity, distinguishing it from other oral blistering conditions. In the mouth, **herpes zoster** initially presents as vesicles that break down quickly, leaving ulcers that heal within 10 to 14 days. The prodromal pain before the rash may be confused with a toothache, leading to unnecessary dental treatment.

The differential diagnosis includes:

- Cellulitis
- Chickenpox
- Cnidaria envenomation
- Contact stomatitis
- Dermatologic manifestations of **herpes** simplex infection
- Ecthyma
- Erysipelas
- Erysipeloid
- Folliculitis
- Human cowpox infections
- Irritant contact dermatitis
- Insect bites
- Paederus dermatitis [20]
- Lichen striatus
- Mucosal candidiasis

Prognosis

Herpes zoster usually resolves within 10 to 15 days in healthy individuals and has an excellent prognosis in younger patients. However, older adults and immunocompromised individuals face significantly higher risks of complications, including severe pain and potentially life-threatening disseminated infections. The principal morbidity is postherpetic neuralgia, which can persist for months and profoundly affect quality of life. Vaccination with the recombinant **zoster** vaccine can reduce the incidence and severity of **herpes zoster**. Ultimately, outcomes are most favorable with early diagnosis and prompt initiation of antiviral therapy.

Complications

Complications include:

- Postherpetic neuralgia
- Bacterial infection
- Ocular complications, including chronic ocular inflammation, vision loss, and debilitating pain
- Ramsay Hunt syndrome
- Meningoencephalitis
- Motor paralysis
- Pneumonitis
- Hepatitis
- Aseptic meningitis
- Peripheral motor neuropathy
- Guillain-Barré syndrome
- Transverse myelitis
- Cardiac arrhythmias
- Urinary retention and hematuria

Deterrence and Patient Education

Prevention of **herpes zoster** focuses on vaccination, specifically the recombinant **zoster** vaccine (Shingrix), which is recommended for adults older than 50 years and immunocompromised individuals, as it significantly reduces the incidence of both **herpes zoster** and postherpetic neuralgia. Patient education should stress prompt medical attention after the appearance of erythematous vesicles and ipsilateral pain. Patients should avoid direct contact with high-risk groups, such as pregnant women, immunocompromised people, and those who have never had chickenpox. Promoting vaccine use and public awareness will help minimize disease burden and improve quality of life.

Enhancing Healthcare Team Outcomes

Shingles is a common infectious disorder in older adults with significant morbidity. The condition has no cure but can be prevented in most patients through vaccination. When eye involvement is present, patients must be referred promptly to an ophthalmologist. Healthcare professionals, including primary care clinicians, nurse practitioners, internists, and pharmacists, should educate patients on the benefits of the vaccine.[21][22]

Review Questions

- Access free multiple choice questions on this topic.
- Click here for a simplified version.
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Figures



Figure 1

Herpes Zoster. **Herpes zoster** presents as a unilateral, dermatomal vesicular eruption on an erythematous base. DermNet New Zealand



Figure 2

Herpes Zoster Rash. **Herpes Zoster**, aka shingles, typically presents with severe pain accompanied by a unilateral rash. The symptoms have a dermatomal distribution. The trunk and the face are most commonly involved. Contributed by S Bhimji, MD



Figure 6

Herpes Zoster Vesicles. **Herpes zoster** commonly presents as grouped vesicles on an erythematous base in a unilateral dermatomal distribution. Contributed by S Munakomi, MD

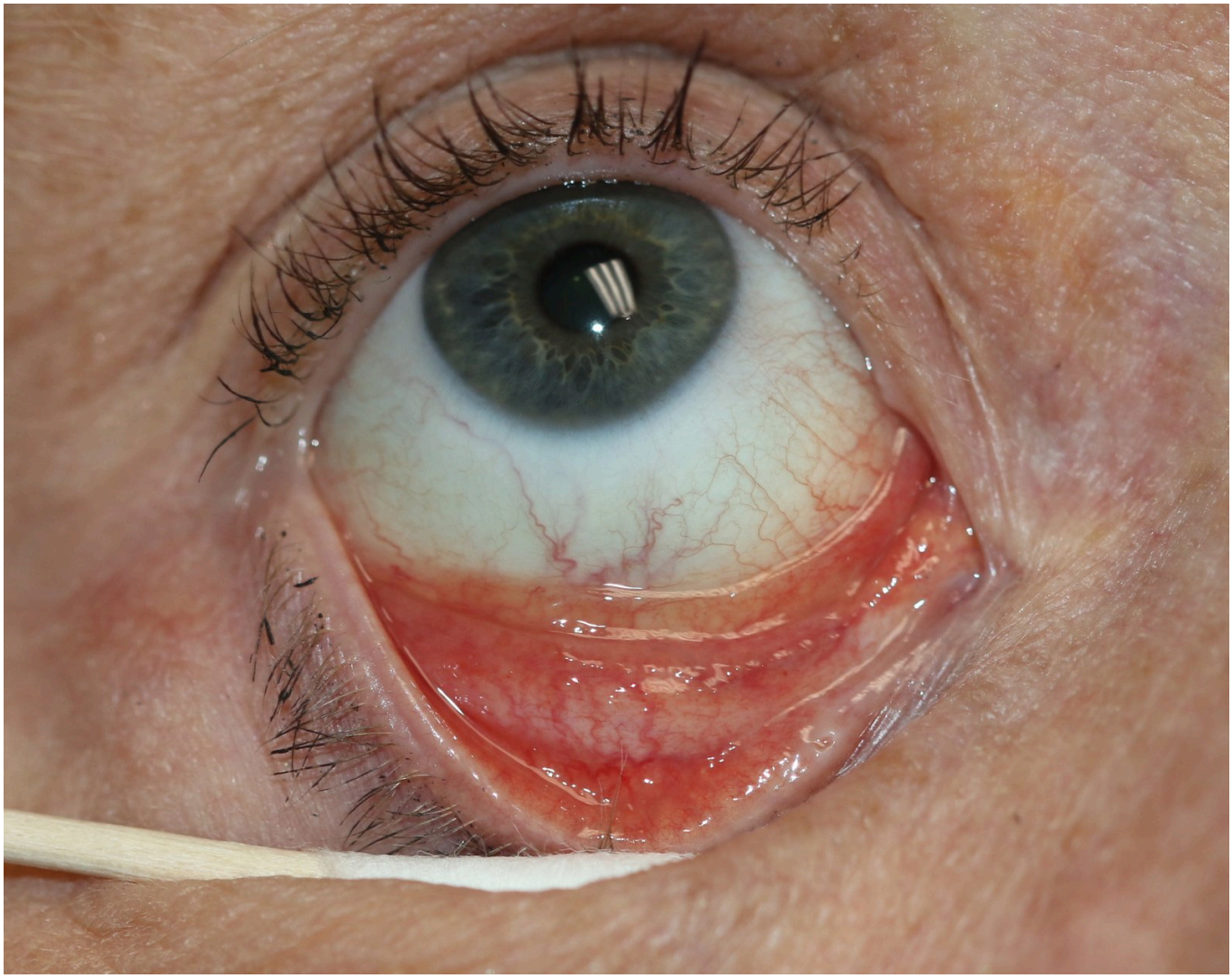


Figure 3

Follicular Conjunctivitis. Inflammation is noted with viral infections like **herpes zoster**, Epstein-Barr virus infection, infectious mononucleosis, and chlamydial infections, as well as in reaction to topical medications and molluscum contagiosum. Follicular conjunctivitis has been described in patients with COVID-19. The inferior and superior tarsal conjunctiva and the fornices show gray-white elevated swellings about 0.5 to 1 mm in diameter and have a velvety appearance. Contributed by BCK Patel, MD, FRCS



Figure 4

Causes of Epiphora. The conditions that can cause epiphora include **herpes zoster** with keratitis (A), lacrimal mucocele (B), corneal calcific keratopathy (C), floppy eyelid syndrome (D), kissing puncta syndrome (E), and pemphigoid disease with trichiasis and obliteration of puncta (F). Contributed by BCK Patel, MD, FRCS



Figure 5

Herpes Zoster Ophthalmicus With Hutchinson Sign. **Herpes Zoster**, aka shingles, can involve the ophthalmic division of the trigeminal nerve, resulting in **herpes zoster** ophthalmicus. The Hutchinson sign, or herpetic lesions in the nasal area, may precede this condition. DermNet New Zealand

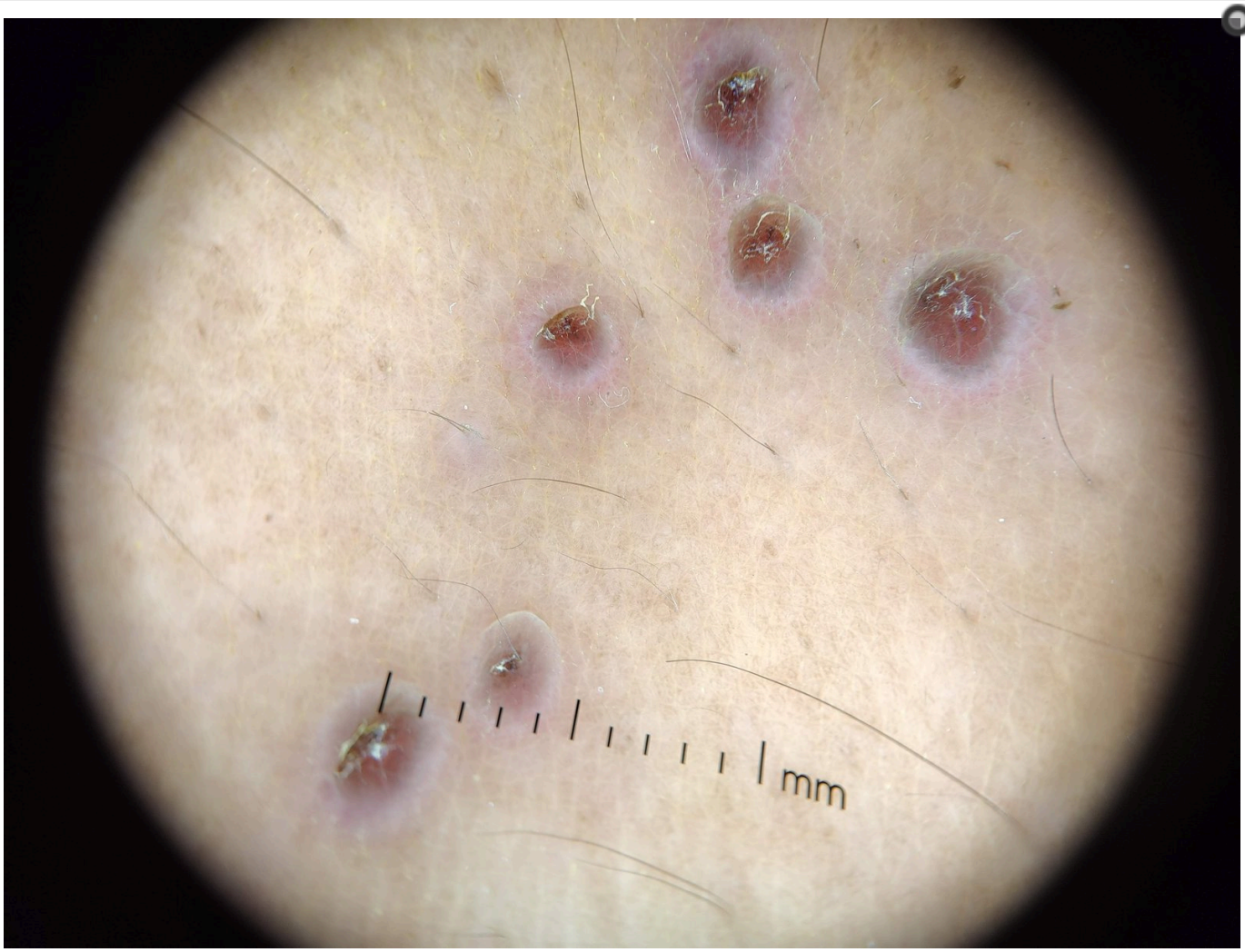


Figure 7

Dermoscopic Halo Sign in **Herpes Zoster**. Dermoscopy of **herpes zoster** may show crusted vesicles with central brown dots surrounded by erythema, creating the characteristic halo sign. Contributed by M Shah, MD

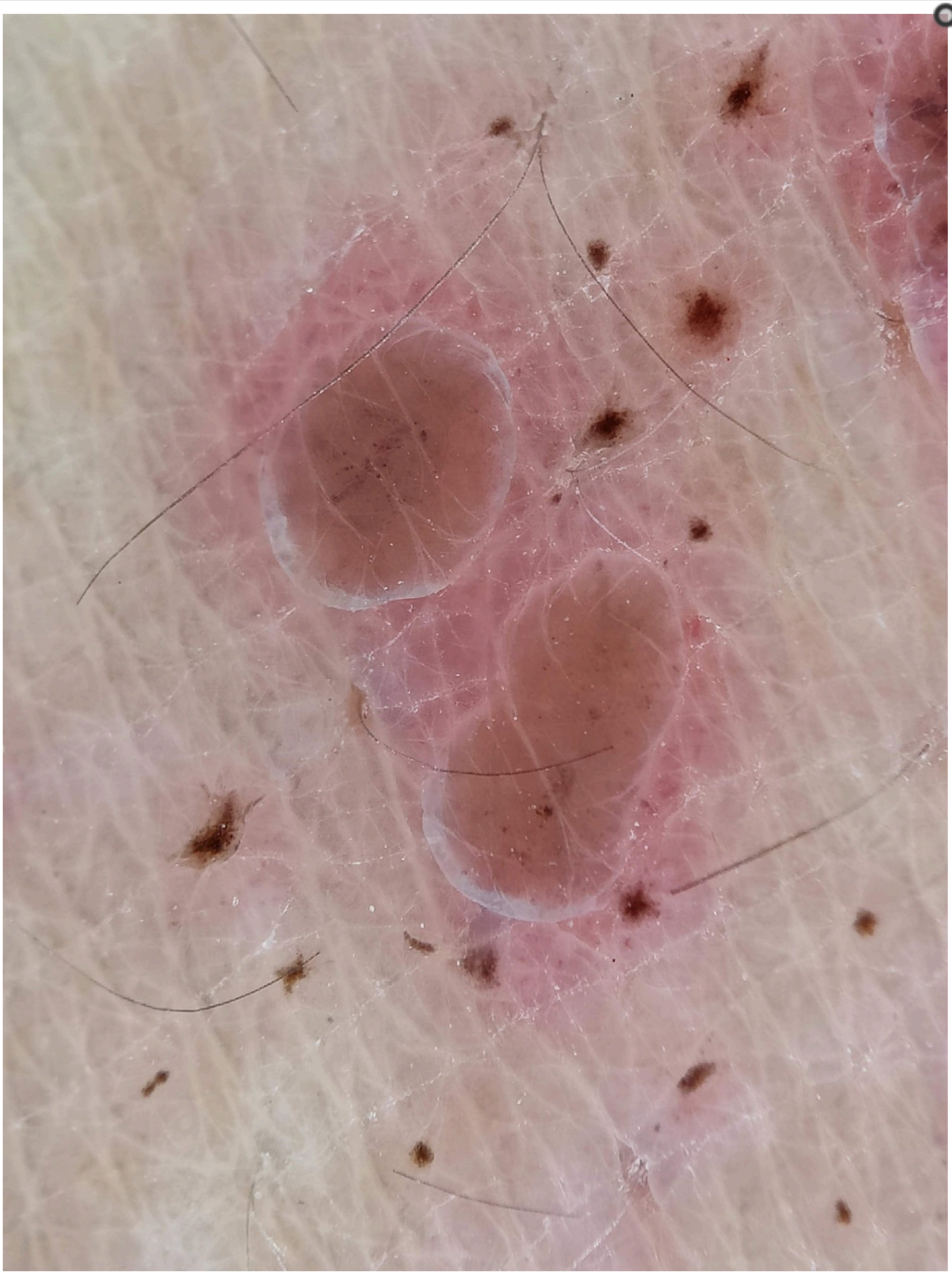


Figure 8

Dermoscopic Vesicles in **Herpes Zoster**. Dermoscopy of **herpes zoster** may show round, cloudy white polyglobular structures corresponding to grouped vesicles, along with central brown dots that correlate with crusted vesicular centers. Contributed by M Shah, MD

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